

BUSINESS REQUIREMENTS DOCUMENT

# Pediatric Care Network

## Intelligent Specialist Matching & Continuity of Care Platform

*Right Child. Right Expert. Right Time. Better Outcomes.*

| Field            | Detail                                                           |
|------------------|------------------------------------------------------------------|
| Document Title   | Business Requirements Document — Pediatric Care Network Platform |
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| Prepared By      | Business Analyst                                                 |
| Business Sponsor | Dr. Prabhu, Chief Pediatrician                                   |
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*This document is a controlled deliverable. Requirements are baselined on approval; subsequent changes are subject to formal change control.*

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# 1. Document Control

## 1.1 Revision History

| Ver | Date        | Author           | Change Summary                                       |
|-----|-------------|------------------|------------------------------------------------------|
| 0.1 | 24-Aug-2026 | Business Analyst | Initial draft derived from the executive vision deck |

## 1.2 Approvals

This document requires sign-off from each of the following before the requirements baseline is established and build commences.

| Role                                  | Name | Signature | Date |
|---------------------------------------|------|-----------|------|
| Business Sponsor / Chief Pediatrician | TBD  |           |      |
| Head of Clinical Operations           | TBD  |           |      |
| CIO / Head of IT                      | TBD  |           |      |
| Data Protection Officer               | TBD  |           |      |
| Quality & Compliance Head (NABH)      | TBD  |           |      |
| Delivery Manager                      | TBD  |           |      |

## 1.3 Distribution List

| Recipient Group             | Purpose                                                           |
|-----------------------------|-------------------------------------------------------------------|
| Clinical Governance Board   | Review and approval of clinical rules and decision-support scope  |
| Hospital Leadership         | Scope, phasing and investment approval                            |
| IT & Integration Team       | Technical feasibility and integration planning                    |
| Legal & Data Protection     | Consent model, children's data processing, repository publication |
| Delivery / Engineering Team | Build input and estimation                                        |

# 2. Executive Summary

The Pediatric Care Network is a cloud-based clinical platform that ensures every child entering the hospital is routed to the most appropriate pediatric specialist at the earliest possible point in their care journey, with qualified clinician oversight at every step.

The platform is explicitly not a diagnostic engine and does not replace clinical judgement. It is a decision-support and care-coordination system with six core capabilities:

1. Capture structured symptom, vitals and clinical assessment data at the point of intake.
2. Recommend the most suitable pediatric specialist based on condition, urgency, specialist experience and historical outcome data — for a clinician to confirm, override or escalate.
3. Maintain a single longitudinal digital record per child covering clinical notes, laboratory results, imaging, media, consents, prescriptions and discharge summaries.

4. Continuously evaluate every open case against five clinical checkpoints and escalate when a case stalls.
5. Give hospital leadership real-time operational visibility across departments and, in later phases, across the facility network.
6. Convert closed cases into a de-identified, searchable knowledge repository that improves future referral accuracy and supports medical education.

**Primary business outcome:** reduce time-to-right-specialist, reduce complications and readmissions arising from delayed or misrouted referrals, and eliminate cases that fall through the cracks between departments.

Delivery is proposed in three phases. Phase 1 establishes the end-to-end clinical journey, the master health record, the checkpoint engine and single-facility leadership dashboards. Phase 2 adds the parent-facing experience, teleconsultation, multidisciplinary collaboration, multi-facility consolidation and the knowledge repository. Phase 3 introduces advanced predictive capability and the medical-education module.

## 3. Business Context & Problem Statement

### 3.1 Current State

Several hundred children present daily across OPD, Emergency, inpatient wards, PICU and NICU. Presentations range from routine childhood illness to rare conditions requiring highly sub-specialised intervention.

The bottleneck identified by the business sponsor is specific and deliberate:

**Problem statement:** The difficulty is not finding a pediatrician. The difficulty is finding the right pediatric expert at the right time.

### 3.2 Pain Points

| Ref | Pain Point                                                                                       | Business Impact                                                                        |
|-----|--------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------|
| P1  | Referral routing depends on the individual knowledge of whoever is on duty                       | Inconsistent routing; the outcome varies by shift rather than by clinical need         |
| P2  | Specialist experience and outcome history are not visible at the point of referral               | Referrals made on availability rather than suitability                                 |
| P3  | History and prior investigations are fragmented across paper, PACS, LIS and departmental systems | Repeat investigations, delayed decisions, avoidable radiation exposure and blood draws |
| P4  | No systematic tracking of whether a referral was actually acted upon                             | Cases stall silently; follow-ups are missed                                            |
| P5  | Multidisciplinary discussion is ad hoc and undocumented                                          | Decisions are not auditable; clinical rationale is lost                                |
| P6  | Parents lack visibility into who is treating their child and why                                 | Low trust, repeated queries, friction at consent points                                |
| P7  | Leadership has no live operational view across departments or facilities                         | Reactive management; load imbalance detected late                                      |

| Ref | Pain Point                                                       | Business Impact                                 |
|-----|------------------------------------------------------------------|-------------------------------------------------|
| P8  | Clinical learning from treated cases is not captured or reusable | Institutional knowledge leaves with individuals |

### 3.3 Desired Future State

A single platform in which every child is assessed correctly, guided intelligently, connected to the right specialist without delay, treated according to evidence-based protocol, and monitored until complete recovery — with parents informed throughout and their right to a second opinion explicitly supported.

### 3.4 Guiding Principles

The following principles were stated by the sponsor as non-negotiable. Each carries a direct and testable design implication.

| ID  | Principle                                                                         | Design Implication                                                                                                             |
|-----|-----------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------|
| GP1 | <b>Human in the loop</b> — the system recommends, a clinician decides             | No automatic assignment without clinician confirmation; override always available and always recorded with a structured reason |
| GP2 | <b>Data in action</b> — recommendations grounded in real history and outcomes     | Every recommendation must present its supporting evidence, not merely a ranked name                                            |
| GP3 | <b>Transparency to parents</b> — parents see relevant experience and outcome data | Parent-facing views must be curated, accurate, risk-adjusted and non-misleading                                                |
| GP4 | <b>Second opinion is a right</b>                                                  | The second-opinion request must be a first-class, frictionless workflow that the system never discourages                      |
| GP5 | <b>Continuity until closure</b>                                                   | A case is not complete until clinically resolved and signed off; the system chases, not human memory                           |
| GP6 | <b>Every case teaches</b>                                                         | Closed cases feed the knowledge base and the recommendation weighting                                                          |

## 4. Business Objectives & Success Metrics

Each objective is paired with a measurable KPI. Baseline values must be established during discovery; without them, objectives BO2, BO3 and BO5 cannot be verified at acceptance. This is raised as open question OQ-01.

| ID  | Business Objective                                      | KPI                                       | Baseline | Target              |
|-----|---------------------------------------------------------|-------------------------------------------|----------|---------------------|
| BO1 | Reduce time from presentation to the correct specialist | Average referral time                     | TBD      | < 30 minutes        |
| BO2 | Improve referral accuracy                               | % referrals not re-routed within 48 hours | TBD      | +24% over baseline  |
| BO3 | Improve clinical outcomes                               | Recovery rate / complication rate         | TBD      | +18% improvement    |
| BO4 | Eliminate missed follow-ups                             | Count of overdue follow-ups               | TBD      | Zero missed per day |
| BO5 | Reduce readmissions                                     | 30-day readmission rate                   | TBD      | -10% year on year   |

| ID  | Business Objective                           | KPI                                             | Baseline | Target       |
|-----|----------------------------------------------|-------------------------------------------------|----------|--------------|
| BO6 | Establish a single source of truth per child | % active cases with a complete digital record   | TBD      | > 95%        |
| BO7 | Improve leadership responsiveness            | Median time from alert raised to action logged  | TBD      | < 60 minutes |
| BO8 | Build institutional knowledge                | Closed, consented cases published to repository | 0        | 100%         |
| BO9 | Increase parent confidence                   | Parent satisfaction on communication            | TBD      | > 4.5 / 5    |

## 5. Scope

### 5.1 In Scope — Phase 1 (MVP)

- Child and guardian registration, identity resolution and consent capture
- Symptom intake and structured clinical assessment with age-aware vitals
- Specialist recommendation engine with mandatory clinician confirmation
- Referral routing, acceptance and in-person consultation scheduling
- Master Health Record — notes, labs, imaging, prescriptions, consents, discharge summaries
- Diagnosis coding and personalised treatment plan capture
- Follow-up scheduling, monitoring and clinician-signed case closure
- Clinical Intelligence checkpoint engine and escalation queues
- Executive Command Center dashboards for a single facility
- Role-based access control, break-glass access and full audit trail
- Master data administration — specialties, departments, specialists, protocols

### 5.2 In Scope — Phase 2

- Parent / guardian portal (web and mobile)
- Virtual consultation (teleconsultation)
- Multidisciplinary team collaboration workspace
- Second-opinion request workflow
- Multi-hospital rollout with consolidated network view
- Knowledge Repository — de-identified case search, imaging library, advanced filters
- Learning loop feeding closed-case outcomes into recommendation weighting

### 5.3 In Scope — Phase 3

- Advanced pattern recognition and early complication detection models
- Predictive analytics for bed and load forecasting
- Medical education module — curated teaching sets and rare-case collections
- External referral network beyond the hospital group

## 5.4 Out of Scope

The following are explicitly excluded from all phases of this initiative.

- Autonomous diagnosis or treatment decisions without clinician sign-off
- Billing, revenue cycle management, insurance claims and TPA processing
- Pharmacy inventory and supply chain management
- HR, payroll and staff rostering — roster data is consumed, not managed
- Replacement of the existing HIS, LIS or RIS-PACS; the platform integrates with them
- Adult care pathways
- Direct-to-consumer patient acquisition or marketplace functionality

## 6. Stakeholders & User Personas

### 6.1 Stakeholder Register

| Stakeholder                     | Interest                                                  | Involvement                            |
|---------------------------------|-----------------------------------------------------------|----------------------------------------|
| Chief Pediatrician (Sponsor)    | Outcome quality, referral accuracy, network visibility    | Approves scope; owns clinical rules    |
| Pediatricians / Care Team       | Fast, accurate routing with low documentation burden      | Primary users; UAT participants        |
| Pediatric Sub-specialists       | Relevant referrals with complete context on arrival       | Primary users; UAT participants        |
| Nursing & Care Coordinators     | Task clarity and reliable follow-up tracking              | Primary users                          |
| Hospital Leadership / COO       | Capacity, load balance, throughput, safety                | Dashboard consumers                    |
| Quality & Safety (NABH)         | Complication tracking, protocol adherence, audit evidence | Requirements input; audit sign-off     |
| Parents / Guardians             | Clarity, speed, trust and choice                          | End users (Phase 2); consent providers |
| IT & Integration Team           | Interoperability, uptime, security                        | Delivery and ongoing support           |
| Data Protection Officer / Legal | Lawful processing of children's health data               | Mandatory sign-off gate                |
| Medical Education / Academics   | Teaching cases and research access                        | Repository consumers (Phase 3)         |

### 6.2 User Personas

| Persona    | Role                   | Primary Need                                                                          |
|------------|------------------------|---------------------------------------------------------------------------------------|
| Dr. Anitha | General Pediatrician   | "Tell me who this child should see, and show me why."                                 |
| Dr. Raghav | Pediatric Cardiologist | "Don't send me cases outside my scope; when you do, send the full history with them." |
| Sr. Meera  | Care Coordinator       | "Show me every child waiting on a step, and who owns it."                             |

| Persona           | Role                | Primary Need                                                                            |
|-------------------|---------------------|-----------------------------------------------------------------------------------------|
| Dr. Prabhu        | Chief Pediatrician  | "One live screen across all departments — and tell me what needs my attention now."     |
| Mr. & Mrs. Sharma | Parents / Guardians | "Who is treating my child, why them, what happens next, and can I get another opinion?" |
| Priya             | Quality Officer     | "Evidence that protocol was followed, with a name and timestamp on every decision."     |

## 7. Business Process — The Child Journey

The care journey comprises ten defined stages. Every open case sits in exactly one stage at any point in time, and every stage transition is timestamped and attributed to a named user.

| #  | Stage                            | Trigger                                  | Owner                                 | Exit Criteria                                                       |
|----|----------------------------------|------------------------------------------|---------------------------------------|---------------------------------------------------------------------|
| 1  | <b>Registration</b>              | Child arrives or pre-registers           | Front desk                            | Child and guardian identity and consent captured                    |
| 2  | <b>Symptoms</b>                  | Registration complete                    | Parent + nurse                        | Structured symptom set and vitals recorded                          |
| 3  | <b>Clinical Assessment</b>       | Symptoms captured                        | Pediatrician / care team              | Assessment notes, provisional impression and urgency grade recorded |
| 4  | <b>Specialist Recommendation</b> | Assessment complete                      | System recommends; clinician confirms | Specialist confirmed, or overridden with a documented reason        |
| 5  | <b>Consultation</b>              | Referral accepted                        | Specialist                            | Consultation conducted and documented (in-person or virtual)        |
| 6  | <b>Diagnosis</b>                 | Consultation and investigations complete | Specialist                            | Confirmed diagnosis coded and recorded                              |
| 7  | <b>Treatment</b>                 | Diagnosis confirmed                      | Specialist + care team                | Treatment plan created and treatment initiated                      |
| 8  | <b>Recovery</b>                  | Treatment in progress                    | Care team                             | Progress monitored against plan milestones                          |
| 9  | <b>Follow-up</b>                 | Discharge or plan milestone              | Care coordinator                      | Scheduled follow-ups completed and outcomes recorded                |
| 10 | <b>Case Closed</b>               | Clinical resolution                      | Specialist sign-off                   | Child confirmed recovered or stable; case archived                  |

**Escalation overlay:** any case exceeding the SLA for its current stage is automatically flagged (At Risk, then Needs Attention) and surfaced in the Alerts Center regardless of which stage it occupies.

## 8. Functional Requirements



Requirements are grouped into thirteen functional modules. Priority is expressed as M (Must have — MVP), S (Should have) and C (Could have). Phase allocation is noted where it differs from Phase 1.

## 8.1 Registration, Identity & Consent

| ID         | Requirement                                                                                                                                                                                     | Pri |
|------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----|
| FR-REG-001 | Register a child with demographics (name, date of birth, gender, blood group) and auto-generate a unique persistent Patient ID.                                                                 | M   |
| FR-REG-002 | Link one or more guardians to a child record with relationship type and a legal-guardian flag.                                                                                                  | M   |
| FR-REG-003 | Prevent duplicate registration through fuzzy matching on name, date of birth and guardian contact; present merge candidates to an authorised user.                                              | M   |
| FR-REG-004 | Capture and version digital consents (treatment, data processing, imaging and media capture, repository publication, teleconsultation) with timestamp, capturing user and consent text version. | M   |
| FR-REG-005 | Allow consent withdrawal at any time and propagate withdrawal to all downstream use, including removal from the repository publication queue.                                                   | M   |
| FR-REG-006 | Support emergency registration with a minimum dataset, flagged for mandatory completion within a configurable window.                                                                           | M   |
| FR-REG-007 | Integrate with the existing HIS so that the platform Patient ID reconciles to the hospital MRN.                                                                                                 | M   |
| FR-REG-008 | Support ABHA / national health ID linkage where the guardian opts in.                                                                                                                           | S   |

## 8.2 Symptom Intake & Clinical Assessment

| ID         | Requirement                                                                                                                                                                              | Pri |
|------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----|
| FR-ASM-001 | Capture presenting complaints using a structured, age-aware symptom taxonomy with a free-text supplement.                                                                                | M   |
| FR-ASM-002 | Record vitals (temperature, heart rate, respiratory rate, SpO <sub>2</sub> , blood pressure, weight, height, head circumference) with age-appropriate reference ranges displayed inline. | M   |
| FR-ASM-003 | Automatically flag any vital sign falling outside the age-appropriate threshold.                                                                                                         | M   |
| FR-ASM-004 | Capture clinical assessment notes, examination findings and provisional impression.                                                                                                      | M   |
| FR-ASM-005 | Assign an urgency grade (Emergency / Urgent / Routine) that drives the downstream referral SLA.                                                                                          | M   |
| FR-ASM-006 | Surface the child's complete prior history — past cases, diagnoses, allergies, medications and prior investigations — inline during assessment.                                          | M   |
| FR-ASM-007 | Support voice-to-text and templated note entry to reduce documentation time.                                                                                                             | C   |

### 8.3 Intelligent Specialist Recommendation Engine

This module carries the highest clinical-safety weight in the platform. Every requirement below is subject to clinical governance sign-off before release.

| ID         | Requirement                                                                                                                                                                                                                                                       | Pri |
|------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----|
| FR-REC-001 | Generate a ranked list of recommended specialists based on presenting condition, urgency, required sub-specialty, specialist availability, specialist experience in that condition, and historical outcome data.                                                  | M   |
| FR-REC-002 | Display, for each recommendation, the supporting rationale — matched condition, case volume in that condition, outcome history, current load and next availability.                                                                                               | M   |
| FR-REC-003 | Never auto-assign. A recommendation becomes a referral only upon explicit clinician confirmation.                                                                                                                                                                 | M   |
| FR-REC-004 | Allow the clinician to override any recommendation and select a different specialist; a structured override reason is mandatory.                                                                                                                                  | M   |
| FR-REC-005 | Log every recommendation, its inputs and rationale, the clinician decision and the eventual outcome, for audit and model evaluation.                                                                                                                              | M   |
| FR-REC-006 | Support recommendation of a multidisciplinary group where the condition spans specialties.                                                                                                                                                                        | S   |
| FR-REC-007 | Where no confident match exists, return “no confident recommendation” and route to a designated senior clinician rather than returning a low-confidence guess.                                                                                                    | M   |
| FR-REC-008 | Support the defined pediatric specialties — Cardiology, Neurology, Nephrology, Urology, Gastroenterology, Endocrinology, Pulmonology, Pediatric Surgery, Neonatology and PICU — and allow further specialties to be added by configuration without a code change. | M   |
| FR-REC-009 | Make recommendation logic (rules, weights, thresholds) configurable by authorised clinical governance users, with versioning and effective dates.                                                                                                                 | M   |
| FR-REC-010 | Maintain and report a referral accuracy metric — recommendation accepted and not re-routed within 48 hours — by specialty and by model version.                                                                                                                   | M   |

**BA note:** FR-REC-003 and FR-REC-004 together constitute the platform's primary clinical risk control. They are also the basis of any argument that the platform is decision support rather than a regulated medical device. They must not be relaxed for convenience during build.

### 8.4 Referral & Consultation Management

| ID         | Requirement                                                                                                       | Pri |
|------------|-------------------------------------------------------------------------------------------------------------------|-----|
| FR-REF-001 | Create a referral to the confirmed specialist with the full clinical context attached.                            | M   |
| FR-REF-002 | Notify the receiving specialist in real time, in-app and via a configurable channel, with urgency-based priority. | M   |
| FR-REF-003 | Allow the specialist to accept, or to decline with a mandatory reason that triggers re-recommendation.            | M   |
| FR-REF-004 | Track the referral against the SLA for its urgency grade; a breach raises an escalation.                          | M   |
| FR-REF-005 | Schedule the consultation (slot, location, modality) and notify the guardian.                                     | M   |

| ID         | Requirement                                                                                                             | Pri     |
|------------|-------------------------------------------------------------------------------------------------------------------------|---------|
| FR-REF-006 | Support virtual consultation with secure video, in-session note capture, and recording only where explicitly consented. | S — Ph2 |
| FR-REF-007 | Record the consultation outcome and next action — investigate, admit, treat, refer onward or discharge.                 | M       |
| FR-REF-008 | Maintain the full referral chain when a child moves between specialists, preserving the reason at each hop.             | M       |

## 8.5 Multidisciplinary Collaboration

| ID         | Requirement                                                                                            | Pri     |
|------------|--------------------------------------------------------------------------------------------------------|---------|
| FR-MDT-001 | Convene a multidisciplinary discussion on a case, inviting named specialists.                          | S — Ph2 |
| FR-MDT-002 | Present the complete history, prior data and investigations to all participants in a shared case view. | S — Ph2 |
| FR-MDT-003 | Record the decision, the participating clinicians, the evidence considered and the rationale.          | S — Ph2 |
| FR-MDT-004 | Record that the guardian was informed of the outcome and capture their acknowledgement.                | S — Ph2 |
| FR-MDT-005 | Link the multidisciplinary decision to the resulting treatment plan.                                   | S — Ph2 |

## 8.6 Diagnosis, Treatment Plan & Recovery

| ID         | Requirement                                                                                                                 | Pri     |
|------------|-----------------------------------------------------------------------------------------------------------------------------|---------|
| FR-TRT-001 | Record confirmed diagnosis using a standard coding system (ICD-10 / ICD-11), supporting multiple and provisional diagnoses. | M       |
| FR-TRT-002 | Create a personalised treatment plan with objectives, interventions, medications, milestones and expected review dates.     | M       |
| FR-TRT-003 | Link the treatment plan to an applicable evidence-based protocol from the protocol library where one exists.                | M       |
| FR-TRT-004 | Flag deviations from the linked protocol and require a documented reason.                                                   | S       |
| FR-TRT-005 | Generate a parent-readable version of the care plan in plain language.                                                      | S — Ph2 |
| FR-TRT-006 | Track progress against plan milestones and record recovery status.                                                          | M       |
| FR-TRT-007 | Record complications with type, severity, detection date and action taken.                                                  | M       |
| FR-TRT-008 | Support amendment of the treatment plan with full version history retained.                                                 | M       |

## 8.7 Master Health Record

| ID         | Requirement                                                                                                                                                          | Pri |
|------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----|
| FR-MHR-001 | Maintain one longitudinal record per child spanning all cases, episodes and departments.                                                                             | M   |
| FR-MHR-002 | Store and render clinical notes, lab results, ultrasound, CT, MRI, X-ray, photographs, surgery images, videos, consent forms, prescriptions and discharge summaries. | M   |

| ID         | Requirement                                                                                                                                  | Pri |
|------------|----------------------------------------------------------------------------------------------------------------------------------------------|-----|
| FR-MHR-003 | Provide four views of the record: Overview, Timeline (chronological), Documents (by type) and Reports.                                       | M   |
| FR-MHR-004 | Display a persistent record header showing Patient ID, age, blood group, allergies and active clinical alerts on every screen.               | M   |
| FR-MHR-005 | Ingest imaging from RIS/PACS via DICOM and laboratory results via HL7/FHIR rather than relying on manual upload.                             | M   |
| FR-MHR-006 | Support in-browser preview of images and documents without requiring download.                                                               | M   |
| FR-MHR-007 | Enforce record immutability — corrections are made as new versions with an addendum reason; nothing is silently overwritten or hard-deleted. | M   |
| FR-MHR-008 | Log every access to a child record (user, time, artefact, source) and make the access log reportable.                                        | M   |
| FR-MHR-009 | Export the complete record as a portable, standards-compliant bundle on authorised request.                                                  | S   |
| FR-MHR-010 | Enforce media-capture consent before any photograph or video can be attached to a record.                                                    | M   |

## 8.8 Follow-up, Monitoring & Case Closure

| ID         | Requirement                                                                                                                | Pri |
|------------|----------------------------------------------------------------------------------------------------------------------------|-----|
| FR-FUP-001 | Auto-generate follow-up schedules from the treatment plan and the applicable protocol.                                     | M   |
| FR-FUP-002 | Maintain a work queue of pending follow-ups showing owner, due date and ageing.                                            | M   |
| FR-FUP-003 | Send reminders to guardians ahead of each follow-up through configurable channels.                                         | M   |
| FR-FUP-004 | Escalate an overdue follow-up automatically once a configurable threshold is passed.                                       | M   |
| FR-FUP-005 | Record the follow-up outcome and update recovery status accordingly.                                                       | M   |
| FR-FUP-006 | Require explicit specialist sign-off to close a case, confirming clinical resolution.                                      | M   |
| FR-FUP-007 | Prevent closure while mandatory checkpoints remain incomplete, unless closed under an authorised and documented exception. | M   |
| FR-FUP-008 | Support case re-opening on relapse, linked to the original case.                                                           | M   |

## 8.9 Clinical Intelligence & Escalation Engine

The engine continuously evaluates five checkpoints for every open case:

7. Has the child seen the correct specialist?
8. Has treatment started?
9. Is follow-up completed?
10. Are there any complications?
11. Is the case resolved?

| ID         | Requirement                                                                                                                            | Pri     |
|------------|----------------------------------------------------------------------------------------------------------------------------------------|---------|
| FR-CIE-001 | Evaluate all five checkpoints for every open case on a continuous or scheduled basis.                                                  | M       |
| FR-CIE-002 | Classify each case as On Track, At Risk or Needs Attention according to configurable rules.                                            | M       |
| FR-CIE-003 | Compute a Clinical Intelligence Score representing the proportion of checkpoints actively satisfied across the active case population. | M       |
| FR-CIE-004 | Maintain escalation queues for At Risk Cases, Overdue Follow-ups, Complications Detected and Ready for Closure.                        | M       |
| FR-CIE-005 | Route each escalation to a named owner with an SLA; unacknowledged escalations escalate one level up.                                  | M       |
| FR-CIE-006 | Record acknowledgement and the action taken against every escalation, closing the loop.                                                | M       |
| FR-CIE-007 | Allow clinical governance to configure checkpoint rules, thresholds and SLAs without a code release.                                   | M       |
| FR-CIE-008 | Apply early-complication detection patterns derived from historical outcome data.                                                      | C — Ph3 |

**BA note:** alert fatigue is the primary failure mode for engines of this kind. Alert-to-action rate should be tracked as a product KPI from day one of the pilot, and thresholds tuned before wider rollout.

## 8.10 Parent & Guardian Experience

| ID         | Requirement                                                                                                                                                              | Pri     |
|------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------|
| FR-PAR-001 | Show the guardian which specialist is treating their child and why that specialist was selected.                                                                         | S — Ph2 |
| FR-PAR-002 | Display the specialist's relevant experience and outcome data for the child's condition, in a curated, accurate and non-misleading form approved by clinical governance. | S — Ph2 |
| FR-PAR-003 | Show the current care journey stage and what happens next.                                                                                                               | S — Ph2 |
| FR-PAR-004 | Provide a plain-language care plan and appointment schedule.                                                                                                             | S — Ph2 |
| FR-PAR-005 | Provide a one-click second-opinion request that is logged, routed, and never blocked or discouraged by the system.                                                       | S — Ph2 |
| FR-PAR-006 | Allow secure viewing of reports and discharge summaries once released by the clinician.                                                                                  | S — Ph2 |
| FR-PAR-007 | Capture digital consent from the guardian remotely.                                                                                                                      | S — Ph2 |
| FR-PAR-008 | Scope guardian access strictly to their own linked children, with access revocable at any time.                                                                          | M       |
| FR-PAR-009 | Support multilingual content — English plus regional languages.                                                                                                          | C       |

**BA note:** FR-PAR-002 requires careful handling. Publishing individual clinician outcome statistics to families creates a de facto league table, with case-mix bias, legal, HR and defensive-practice consequences. Clinical governance and Legal must jointly define which metrics are shown, at what aggregation, and with what risk adjustment. Tracked as OQ-05.

## 8.11 Executive Command Center

| ID         | Requirement                                                                                                                                                                                                                           | Pri     |
|------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------|
| FR-EXE-001 | Present live KPI tiles for Children Registered, Waiting Patients, Emergency Cases, Specialists Assigned, Average Referral Time, Pending Follow-ups, Recovered, Complications and Readmissions, each with period-over-period variance. | M       |
| FR-EXE-002 | Show department performance including patients today, average referral time, backlog, status indicator and trend.                                                                                                                     | M       |
| FR-EXE-003 | Show a live hospital heat map by area (OPD, Emergency, PICU, NICU, Wards, Imaging) with occupancy or waiting counts banded Low / Moderate / High / Critical.                                                                          | M       |
| FR-EXE-004 | Present an Executive Alerts feed, newest first, with acknowledgement capture.                                                                                                                                                         | M       |
| FR-EXE-005 | Present system-generated leadership recommendations as advisory suggestions requiring human action; these are never auto-executed.                                                                                                    | M       |
| FR-EXE-006 | Provide trend visualisations for referral time, patient volume mix and outcomes.                                                                                                                                                      | M       |
| FR-EXE-007 | Provide a leadership summary showing total active cases, critical cases, average length of stay, bed occupancy and staff on duty.                                                                                                     | M       |
| FR-EXE-008 | Support facility filtering, including a consolidated all-facilities view.                                                                                                                                                             | S — Ph2 |
| FR-EXE-009 | Provide navigation to Patients, Departments, Referrals, Cases, Follow-ups, Quality & Safety, Reports, Analytics, Alerts Center and Settings.                                                                                          | M       |
| FR-EXE-010 | Support drill-down from any KPI tile to the underlying case list.                                                                                                                                                                     | M       |
| FR-EXE-011 | Support scheduled and ad-hoc report export to PDF and Excel, with the data-as-of timestamp printed on the output.                                                                                                                     | M       |
| FR-EXE-012 | Refresh dashboards at a configurable interval and display the last-refreshed time explicitly.                                                                                                                                         | M       |

## 8.12 Knowledge Repository & Learning Loop

| ID         | Requirement                                                                                                                                                  | Pri     |
|------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------|---------|
| FR-KNW-001 | Publish closed cases to the repository in de-identified form, only where consent permits.                                                                    | M — Ph2 |
| FR-KNW-002 | Provide keyword search across diagnosis, symptoms, procedures and clinical notes.                                                                            | M — Ph2 |
| FR-KNW-003 | Provide filters for age, gender, department, disease, procedure, outcome, date and location.                                                                 | M — Ph2 |
| FR-KNW-004 | Support image-based search and browsing across X-ray, MRI, CT, ultrasound, clinical photographs and surgery images.                                          | S — Ph2 |
| FR-KNW-005 | Provide curated collections — Top Diseases, Top Procedures, Complications, Imaging Library, Rare Cases and Best Outcomes.                                    | S — Ph2 |
| FR-KNW-006 | Show repository metrics: total cases, diseases covered, procedures, images and media, and successful-outcome rate.                                           | S — Ph2 |
| FR-KNW-007 | Provide a case-detail view showing presentation, pathway, interventions and outcome, with all identifiers removed.                                           | M — Ph2 |
| FR-KNW-008 | Feed closed-case outcomes back into recommendation weighting, with each model version tracked and its impact on referral accuracy measured before promotion. | S — Ph2 |

| ID         | Requirement                                                                                                                                  | Pri     |
|------------|----------------------------------------------------------------------------------------------------------------------------------------------|---------|
| FR-KNW-009 | Track and report learning metrics: cases learned, insights added, protocols improved, referral accuracy improvement and outcome improvement. | S — Ph2 |
| FR-KNW-010 | Enforce a governance workflow — de-identification validation and clinical review — before any case is published.                             | M — Ph2 |
| FR-KNW-011 | Provide a controlled academic and teaching access role, separate from clinical access.                                                       | C — Ph3 |

**BA note:** De-identification must include detection and redaction of identifiers burned into image pixels on DICOM studies and clinical photographs. Header scrubbing alone is insufficient and is a well-documented leak path. This is called out as a distinct build task, not an assumption.

### 8.13 Administration, Security & Audit

| ID         | Requirement                                                                                                                                                                                | Pri |
|------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----|
| FR-ADM-001 | Enforce role-based access control on least-privilege principles across all roles: pediatrician, specialist, nurse, coordinator, leadership, quality, administrator, guardian and academic. | M   |
| FR-ADM-002 | Maintain master data for specialties, departments, specialist profiles and credentials, facilities, protocols, symptom taxonomy and consent templates.                                     | M   |
| FR-ADM-003 | Maintain specialist profiles including sub-specialty, conditions treated, credentials, availability and outcome history.                                                                   | M   |
| FR-ADM-004 | Provide a break-glass emergency access mechanism granting time-boxed access, alerting immediately and requiring post-hoc review.                                                           | M   |
| FR-ADM-005 | Maintain a tamper-evident audit trail of every create, read, update, delete, consent event, override and escalation, capturing user, timestamp, source and before/after values.            | M   |
| FR-ADM-006 | Enforce configurable data retention aligned to pediatric record regulation, typically retention until the child attains majority plus a statutory period.                                  | M   |
| FR-ADM-007 | Support single sign-on with the hospital identity provider and enforce multi-factor authentication for privileged roles.                                                                   | M   |
| FR-ADM-008 | Provide a configuration console for rules, SLAs, thresholds and notification templates that requires no code deployment.                                                                   | M   |

## 9. Data Requirements

### 9.1 Core Business Entities

Child · Guardian · Consent · Case · Encounter · Assessment · SymptomRecord · Vitals · Recommendation · Referral · Consultation · Diagnosis · TreatmentPlan · Intervention · Complication · FollowUp · Document · MediaAsset · Specialist · Specialty · Department · Facility · Protocol · Checkpoint · Alert · Escalation · KnowledgeCase · AuditEvent

## 9.2 Data Standards

| Domain                        | Standard                               |
|-------------------------------|----------------------------------------|
| Clinical interoperability     | HL7 FHIR R4                            |
| Diagnosis coding              | ICD-10 / ICD-11                        |
| Procedures and clinical terms | SNOMED CT (preferred)                  |
| Laboratory results            | LOINC                                  |
| Imaging                       | DICOM                                  |
| Medications                   | Hospital formulary with RxNorm mapping |

## 9.3 Data Quality Rules

- Mandatory fields are enforced at each stage gate; a stage cannot be exited with incomplete mandatory data.
- Duplicate patient creation is prevented at registration through fuzzy matching (FR-REG-003).
- Referential integrity is maintained between case, referral, diagnosis and treatment plan.
- No hard deletion of clinical data — logical deletion with audit trail only.
- All timestamps are stored in UTC and displayed in facility local time.

## 9.4 De-identification Rules for the Repository

- All direct identifiers are removed from published cases.
- Dates are shifted or generalised to prevent timeline-based re-identification.
- Rare-condition cases are individually screened for re-identification risk before publication.
- Identifiers burned into image pixels on DICOM studies and clinical photographs must be detected and redacted.
- Publication is gated on both valid consent and clinical governance review.

# 10. Non-Functional Requirements

| ID     | Category     | Requirement                                                                                                                                                                                                            |
|--------|--------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| NFR-01 | Availability | 99.9% uptime. Critical paths — registration, assessment, referral and record view — must degrade gracefully rather than fail outright.                                                                                 |
| NFR-02 | Performance  | Page load under 2 seconds; recommendation generated within 3 seconds; dashboard refresh within 5 seconds at peak load.                                                                                                 |
| NFR-03 | Scalability  | Support the target concurrent clinical user count per facility with horizontal scale-out across the full facility network.                                                                                             |
| NFR-04 | Security     | Encryption in transit (TLS 1.3) and at rest (AES-256); secrets held in a managed vault; annual penetration testing with tracked remediation.                                                                           |
| NFR-05 | Privacy      | Compliance with the Digital Personal Data Protection Act 2023, including its heightened obligations for processing children's personal data, plus HIPAA and GDPR alignment where the network operates internationally. |



| ID     | Category         | Requirement                                                                                                                                                    |
|--------|------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------|
| NFR-06 | Auditability     | Immutable, exportable audit log retained for the full regulatory retention period.                                                                             |
| NFR-07 | Usability        | Clinical tasks completable in minimal clicks; the assessment-to-referral flow must be completable on a tablet at the bedside.                                  |
| NFR-08 | Accessibility    | WCAG 2.1 Level AA for all parent-facing interfaces.                                                                                                            |
| NFR-09 | Browser & device | Current Chrome, Edge, Safari and Firefox; responsive tablet layouts; native or progressive web app for the parent experience.                                  |
| NFR-10 | Localisation     | English at launch, with an architecture that supports regional languages without rework.                                                                       |
| NFR-11 | Interoperability | FHIR-based APIs for HIS, LIS, RIS-PACS, ADT and identity integration.                                                                                          |
| NFR-12 | Backup & DR      | Recovery point objective of 15 minutes or less; recovery time objective of 4 hours or less; disaster recovery drills twice yearly.                             |
| NFR-13 | Traceability     | Every requirement traceable to a test case; every clinical decision traceable to a named user.                                                                 |
| NFR-14 | Monitoring       | Application, infrastructure and clinical-SLA monitoring with proactive alerting.                                                                               |
| NFR-15 | Model governance | Any change to recommendation logic requires versioning, offline evaluation, clinical sign-off, post-deployment accuracy monitoring and a tested rollback path. |

## 11. Integration Requirements

| ID     | System                        | Direction      | Purpose                                                    |
|--------|-------------------------------|----------------|------------------------------------------------------------|
| INT-01 | HIS / EMR                     | Bi-directional | Patient demographics, MRN, ADT events, encounters          |
| INT-02 | Laboratory Information System | Inbound        | Laboratory orders and results                              |
| INT-03 | RIS / PACS                    | Inbound        | Imaging studies and radiology reports via DICOM            |
| INT-04 | Identity Provider             | Inbound        | Single sign-on, MFA, role provisioning                     |
| INT-05 | Notification gateway          | Outbound       | SMS, email, messaging and push to guardians and clinicians |
| INT-06 | Video platform                | Bi-directional | Teleconsultation (Phase 2)                                 |
| INT-07 | Staff roster system           | Inbound        | Specialist availability and duty status                    |
| INT-08 | Bed management system         | Inbound        | Occupancy data for the hospital heat map                   |
| INT-09 | National health ID (ABDM)     | Bi-directional | ABHA linkage and consented record exchange                 |
| INT-10 | Data warehouse / BI           | Outbound       | Analytics and regulatory reporting                         |

## 12. Regulatory & Compliance Requirements

| ID     | Requirement                                                                                                                                                                                                                                  |
|--------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| REG-01 | Comply with the Digital Personal Data Protection Act 2023, including verifiable parental consent for processing a child's personal data and the prohibition on behavioural tracking or monitoring of children.                               |
| REG-02 | Comply with NABH standards for clinical documentation, consent and quality indicator reporting.                                                                                                                                              |
| REG-03 | Comply with applicable telemedicine practice guidelines for any virtual consultation.                                                                                                                                                        |
| REG-04 | Retain records in accordance with statutory medical-record retention rules applicable to minors.                                                                                                                                             |
| REG-05 | Assess any clinical decision-support capability against applicable Software as a Medical Device classification. The design of "recommendation plus mandatory human confirmation" is the intended risk control and must be validated as such. |
| REG-06 | Maintain a documented validation package (URS, FS, DS, IQ/OQ/PQ and traceability matrix) should the platform be deemed to require formal computer system validation.                                                                         |
| REG-07 | Maintain a data-processing register, a Data Protection Impact Assessment and a documented breach-notification procedure.                                                                                                                     |

## 13. Assumptions, Constraints & Dependencies

### 13.1 Assumptions

| ID | Assumption                                                                                                                                                               |
|----|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| A1 | The hospital operates an existing HIS with an accessible patient master and an API or HL7 interface.                                                                     |
| A2 | Imaging is already digitised in a PACS accessible via DICOM.                                                                                                             |
| A3 | Historical case and outcome data of sufficient volume and quality exists to seed the recommendation engine. Where it does not, Phase 1 operates on clinical rules alone. |
| A4 | Specialists will maintain accurate availability data, directly or through roster integration.                                                                            |
| A5 | Clinicians have tablet or desktop access at the point of care with reliable network connectivity.                                                                        |
| A6 | A clinical governance body will be constituted before build to own rules, protocols and model sign-off.                                                                  |
| A7 | Phase 1 is a single-facility pilot; multi-facility rollout follows a successful pilot.                                                                                   |

### 13.2 Constraints

| ID | Constraint                                                                                                                          |
|----|-------------------------------------------------------------------------------------------------------------------------------------|
| C1 | The platform must not disrupt existing clinical workflows during rollout; parallel running is required throughout the pilot period. |
| C2 | The platform cannot replace the HIS as the system of record for billing and administrative data.                                    |
| C3 | Recommendation quality is bounded by the completeness and quality of historical outcome data.                                       |
| C4 | Any parent-facing display of clinician performance data is subject to legal and HR clearance.                                       |
| C5 | Downtime windows are severely limited in a 24x7 clinical environment.                                                               |

### 13.3 Dependencies

| ID | Dependency                                                             | Owner                       |
|----|------------------------------------------------------------------------|-----------------------------|
| D1 | Integration access and credentials for HIS, LIS and PACS               | Hospital IT                 |
| D2 | Clinical rules, protocol library and specialty taxonomy                | Clinical Governance         |
| D3 | Historical case dataset, cleansed and mapped                           | Clinical + Data team        |
| D4 | Legal and DPO sign-off on the consent model and repository publication | Legal / DPO                 |
| D5 | Baseline KPI measurement prior to build                                | Quality team                |
| D6 | Specialist master data and credentialing records                       | Medical Administration / HR |

### 14. Risk Register

| ID | Risk                                                                                               | Impact | Likelihood | Mitigation                                                                                                                     |
|----|----------------------------------------------------------------------------------------------------|--------|------------|--------------------------------------------------------------------------------------------------------------------------------|
| R1 | Clinicians distrust or ignore recommendations                                                      | High   | Medium     | Explainable rationale on every recommendation; clinician co-design; frictionless override; accuracy metrics published openly   |
| R2 | Recommendation logic trained on biased or incomplete history entrenches existing referral patterns | High   | Medium     | Fairness and case-mix review; rules-first in Phase 1; accuracy monitored by specialty; clinical sign-off gate on every version |
| R3 | Re-identification from repository cases, particularly rare conditions and imaging                  | High   | Medium     | Governance review before publication; burned-in identifier redaction; rare-case risk screening; consent gating                 |
| R4 | Documentation burden slows clinicians and adoption stalls                                          | High   | High       | Time-and-motion study during pilot; templates and voice input; hard cap on mandatory fields per stage                          |
| R5 | Integration delays with legacy HIS and PACS                                                        | High   | Medium     | Early integration spike; contracted vendor support; manual-upload fallback for the pilot                                       |
| R6 | Alert fatigue from the escalation engine                                                           | Medium | High       | Threshold tuning during pilot; role-scoped queues; alert-to-action rate tracked as a product KPI                               |
| R7 | Parent-facing outcome data misread as a clinician league table                                     | High   | Medium     | Clinical governance defines the displayed metrics; risk-adjusted, non-comparative presentation; legal sign-off                 |
| R8 | Scope creep from the vision deck into Phase 1                                                      | Medium | High       | Phased scope baselined in this document; formal change control on all additions                                                |
| R9 | Data privacy non-compliance for children's data                                                    | High   | Low        | DPIA completed before build; DPO in the approval chain; verifiable guardian consent                                            |

## 15. Phased Delivery Plan

| Phase                         | Focus                                                                                                                                     | Indicative Duration |
|-------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------|---------------------|
| <b>Phase 0</b> — Discovery    | Baseline KPI measurement, process mapping, integration assessment, data quality assessment, DPIA, constitution of clinical governance     | 4 – 6 weeks         |
| <b>Phase 1</b> — MVP Pilot    | End-to-end child journey from registration to closure; Master Health Record; Clinical Intelligence engine; single-facility Command Center | 16 – 20 weeks       |
| <b>Phase 2</b> — Extend       | Parent portal, teleconsultation, multidisciplinary workspace, second opinion, multi-facility view, knowledge repository, learning loop    | 12 – 16 weeks       |
| <b>Phase 3</b> — Intelligence | Advanced models, early complication detection, predictive capacity analytics, medical education module                                    | 12+ weeks           |

Durations are indicative and subject to confirmation against a detailed delivery estimate following discovery.

## 16. Business Acceptance Criteria — Phase 1

Phase 1 is accepted when all of the following are demonstrably true in the pilot department, on real clinical cases.

12. A child can be taken end to end from registration through case closure entirely within the platform.
13. Every referral carries either a system recommendation accepted by a clinician or a documented override reason — 100% coverage, no exceptions.
14. Average referral time is measured and demonstrably tracking toward the 30-minute target.
15. No case is closed with incomplete mandatory checkpoints without a documented, authorised exception.
16. The Master Health Record shows a complete, correctly ordered artefact set for at least 95% of pilot cases.
17. The Command Center reflects live pilot-department data, and every executive alert raised has a recorded acknowledgement and action.
18. The audit trail can reconstruct the full decision history of any pilot case on demand for the quality team.
19. Security testing is passed with all high and critical findings remediated.
20. The Data Protection Officer and clinical governance have signed off on the consent model and the recommendation ruleset.

## 17. Open Questions

The following must be resolved before the requirements baseline can be finalised. OQ-01 and OQ-03 have the greatest influence on the shape of the build.

| ID    | Question                                                                                                                                                       | Owner        | Needed By             |
|-------|----------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------|-----------------------|
| OQ-01 | What are the current baseline values for referral time, complication rate, readmission rate and missed follow-ups? Several KPIs are unverifiable without them. | Quality team | Phase 0               |
| OQ-02 | Is Phase 1 a single facility or the full network from day one?                                                                                                 | Sponsor      | Before scope baseline |

| ID    | Question                                                                                                                                                                    | Owner                       | Needed By             |
|-------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------|-----------------------|
| OQ-03 | What historical case volume and outcome data actually exists, and in what condition? This determines whether the recommendation engine launches rules-based or data-driven. | Data + Clinical             | Phase 0               |
| OQ-04 | Which HIS, LIS and PACS products are in place, and what integration interfaces do they expose?                                                                              | Hospital IT                 | Phase 0               |
| OQ-05 | Exactly which specialist experience and outcome metrics may be shown to parents, at what aggregation and with what risk adjustment?                                         | Clinical Governance + Legal | Before Phase 2 design |
| OQ-06 | Does the second-opinion workflow route within the network only, or also externally?                                                                                         | Sponsor                     | Phase 2               |
| OQ-07 | Who constitutes the clinical governance body that owns rules, protocols and model sign-off?                                                                                 | Sponsor                     | Phase 0               |
| OQ-08 | What is the required data retention period for pediatric records in the operating jurisdictions?                                                                            | Legal / DPO                 | Phase 0               |
| OQ-09 | Is the platform expected to be classified as Software as a Medical Device, and does formal validation apply?                                                                | Regulatory                  | Phase 0               |
| OQ-10 | Will repository cases be shared beyond the network for research, academia or publication? Consent language depends on the answer.                                           | Sponsor + Legal             | Phase 2               |
| OQ-11 | What is the budget envelope and the target go-live date?                                                                                                                    | Sponsor                     | Before planning       |
| OQ-12 | Is teleconsultation in scope for the pilot, or strictly Phase 2?                                                                                                            | Sponsor                     | Before scope baseline |

## Appendix A — Requirements Traceability Matrix

Every business objective maps to the requirement groups that deliver it. Each requirement will in turn be traced to test cases during the test design phase.

| Business Objective                     | Supporting Requirements                                                      |
|----------------------------------------|------------------------------------------------------------------------------|
| BO1 — Reduce time to right specialist  | FR-ASM-005; FR-REC-001 to FR-REC-010; FR-REF-001 to FR-REF-004               |
| BO2 — Improve referral accuracy        | FR-REC-002; FR-REC-005; FR-REC-010; FR-KNW-008                               |
| BO3 — Improve clinical outcomes        | FR-TRT-001 to FR-TRT-008; FR-CIE-001 to FR-CIE-007; FR-MDT-001 to FR-MDT-005 |
| BO4 — Eliminate missed follow-ups      | FR-FUP-001 to FR-FUP-008; FR-CIE-004 to FR-CIE-006                           |
| BO5 — Reduce readmissions              | FR-FUP-005; FR-TRT-006; FR-EXE-001                                           |
| BO6 — Single source of truth per child | FR-MHR-001 to FR-MHR-010; INT-01 to INT-03                                   |
| BO7 — Leadership responsiveness        | FR-EXE-001 to FR-EXE-012; FR-CIE-005                                         |
| BO8 — Build institutional knowledge    | FR-KNW-001 to FR-KNW-011                                                     |
| BO9 — Increase parent confidence       | FR-PAR-001 to FR-PAR-009; FR-REG-004                                         |

## Appendix B — Glossary

| Term                        | Definition                                                                             |
|-----------------------------|----------------------------------------------------------------------------------------|
| ABDM / ABHA                 | India's national digital health mission and its associated health account identifier   |
| Case                        | A clinical episode for one child, from presentation through to closure                 |
| Checkpoint                  | One of the five clinical questions evaluated continuously for every open case          |
| Clinical Intelligence Score | The proportion of checkpoints actively satisfied across the active case population     |
| DICOM                       | Digital Imaging and Communications in Medicine — the standard for medical imaging      |
| DPIA                        | Data Protection Impact Assessment                                                      |
| FHIR                        | Fast Healthcare Interoperability Resources — the HL7 interoperability standard         |
| Master Health Record        | The longitudinal consolidated clinical record for one child                            |
| MDT                         | Multidisciplinary Team                                                                 |
| NABH                        | National Accreditation Board for Hospitals and Healthcare Providers                    |
| Referral accuracy           | The percentage of recommendations accepted and not re-routed within 48 hours           |
| SaMD                        | Software as a Medical Device                                                           |
| SLA                         | Service Level Agreement — the time within which a stage or escalation must be actioned |

— End of Document —